

PUBLIC RESEARCH NOTE

Novo Nordisk: *franchise, or falling knife?* — what is the de-rated price assuming — and is Lilly simply ahead?

I ran a full nine-step diligence on Europe's fallen champion. Here's what the ~14× multiple actually prices in, and whether the de-rating is justified by the fundamentals. I'm showing my work — not giving you a call. Data as of 6 July 2026.

- **The market is pricing Novo for near-zero growth.** A reverse-DCF (my calculation) shows today's price implies only ~0–3% annual earnings growth for a decade — for a ~21%/yr historical compounder. Run 5,000 times: fair value clears today's price in ≈52% of futures if cash flow normalises — ≈0% if it doesn't.
- **Quality is elite but turned down across the board in 2025.** Gross margin 84.7%→81.0%, return on capital 88.5%→39.3%, and free cash flow went **negative** in 2024 as capex and an acquisition consumed cash.
- **Lilly leads on momentum.** FY2025 revenue ~\$65bn (+45%) vs Novo ~\$47bn; Zepbound +175% vs Wegovy +31%; tirzepatide beat semaglutide in the only head-to-head.
- **The key risk:** if the decline is structural (not a cyclical dent), the low multiple reflects lower earnings power, not opportunity — and in the one named pharma precedent (PD-1), the head-to-head loser did not regain the lead.
- **The swing question:** does growth re-accelerate off 2026's trough? Genuinely uncertain — and where the whole case is won or lost.

WHAT'S PRICED IN
~0–3% growth /
decade

THIS NOTE
Analysis, not
advice

Key players & terms

New to this sector? Skim these once and the rest reads easily. At heart this is a story of **two companies racing each other** in obesity and diabetes drugs: **Novo Nordisk** and **Eli Lilly**.

Eli Lilly ("Lilly")

The US pharmaceutical giant and Novo's main competitor in diabetes and obesity. The rivalry runs through this whole note.

Semaglutide

Novo's flagship molecule. Sold as **Ozempic** and **Rybelsus** (diabetes) and **Wegovy** (obesity).

CagriSema

Novo's next-generation obesity drug candidate — its main pipeline hope to answer Lilly.

REDEFINE 1 / 2 / 4

The clinical trials of CagriSema. 1 & 2 are the approval trials (both passed); 4 is the head-to-head vs tirzepatide (narrowly lost).

Medicare cut (IRA)

A US law forcing steep price cuts (~71%) on Novo's semaglutide for Medicare patients from 2027.

GLP-1

The class of drugs behind the boom — they mimic a gut hormone that curbs appetite and blood sugar.

Tirzepatide

Lilly's rival molecule. Sold as **Mounjaro** (diabetes) and **Zepbound** (obesity) — it beat Novo's drug head-to-head.

Orforglipron

Lilly's experimental obesity drug in *pill* form — cheaper and easier to scale than injections.

Reverse-DCF

A valuation check that backs out the growth rate the current share price already assumes.

CER

"Constant exchange rates" — growth with currency swings stripped out, so the underlying trend is clearer.

Company snapshot — read this first

€193–197bn Market cap	~14× • ~10.3× P/E • EV/EBITDA	DKK 309bn FY25 sales	59.6% Obesity vol. share
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What it is, in plain English. Novo Nordisk is a *Danish pharmaceutical company* and a global leader in diabetes and obesity care. Most revenue comes from one family of GLP-1 drugs based on **semaglutide** — Ozempic and Rybelsus (diabetes), Wegovy (obesity) — plus legacy insulin and a small rare-disease arm. It is **vertically integrated and foundation-controlled** (a patient, through-cycle owner; ordinary shareholders cannot force a sale), earning high margins on patent-protected GLP-1 drugs, mostly in the **US (56% of revenue)**.

Identity & market (6 July 2026)

Listing	NOVO-B • NVO (ADR 1:1)
Price NOVO-B / ADR	DKK 324.0 / \$50.43
52-wk range (NOVO-B)	224.3 – 464.6
Market cap / EV	€193–197bn / €205bn
Control	Foundation ~77% votes / ~28% economics
CEO	M. Doustdar (since 2025)

Business mix — FY2025

BY SEGMENT

Diabetes 67%

Obesity 27%

6%

BY GEOGRAPHY

US 56%

International 44%

Still ~⅔ diabetes by revenue; obesity is the growth engine (+31% CER). Dividend yield ~3.5% (payout ~50%).

Five-year quality signature — every metric turned down together in 2025 (shaded)

DKK M / %	2021	2022	2023	2024	2025	TREND
Net sales (CAGR 21.7%)	140,800	176,954	232,261	290,403	309,064	
Gross margin	83.2%	83.9%	84.6%	84.7%	81.0%	
Operating margin	41.7%	42.3%	44.2%	44.2%	41.3%	
ROIC (disclosed)	69.0%	73.6%	88.5%	63.9%	39.3%	
Cash conversion (FCF/NP)	61%	103%	82%	-15%	27.6%	

THE TENSION THE DILIGENCE TACKLES

"Branded-obesity volume leader (59.6%) in a market that grew ~104% in 2025 — yet it lost the only head-to-head to Lilly (25.5% vs 23.0%) and, for the first time, every quality metric turned down together."

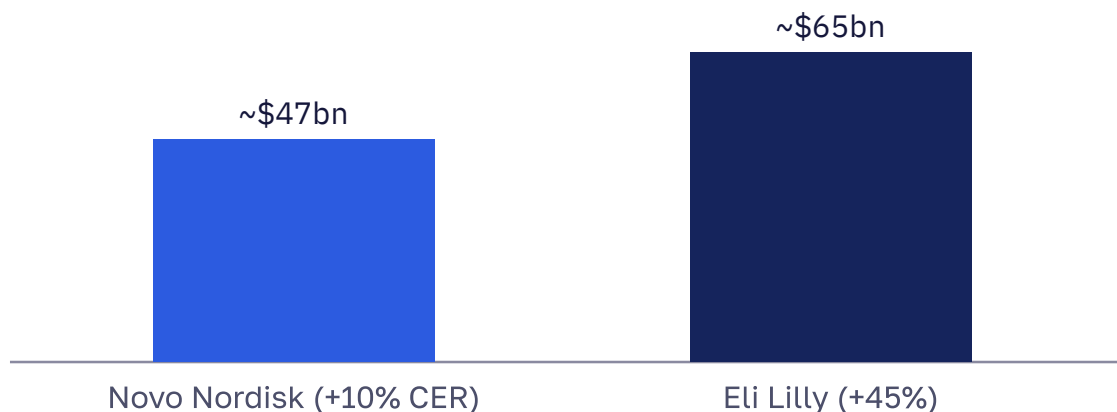
The question: a cyclical dent, or a structural decline?

The business, and who leads the market

By revenue Novo is still *two-thirds a diabetes company* (~67% diabetes, ~27% obesity, ~6% rare disease), with obesity the growth engine, and almost everything riding on one molecule – semaglutide (Ozempic, Rybelsus, Wegovy) – though the pipeline is broadening beyond it (notably amycretin). The US is 56% of revenue, so US pricing is the dominant variable. It is foundation-controlled: the Novo Nordisk Foundation holds ~77% of the votes on ~28% of the economics – a patient, through-cycle owner (the Danish foundation-compounder model), though one that limits minority-holder leverage.

Branded GLP-1 obesity is a secular, ~100%-growing market dominated by Novo and Eli Lilly. Novo holds the volume lead (59.6% branded share), but on momentum Lilly is ahead. The next battleground is oral: Novo's Wegovy pill is a peptide (higher efficacy, harder to scale); Lilly's orforglipron is a small molecule (lower efficacy ~12.4%, but far cheaper to scale). A US Medicare price cut (~71%, from January 2027) is a real headwind.

Exhibit 1: Lilly is bigger and growing far faster – the momentum has shifted



Source: Novo Nordisk AR 2025 (~\$47bn from DKK 309.1bn, FX-converted); Eli Lilly FY2025 results (SEC / IR, \$65.2bn). Retrieved 6 July 2026.

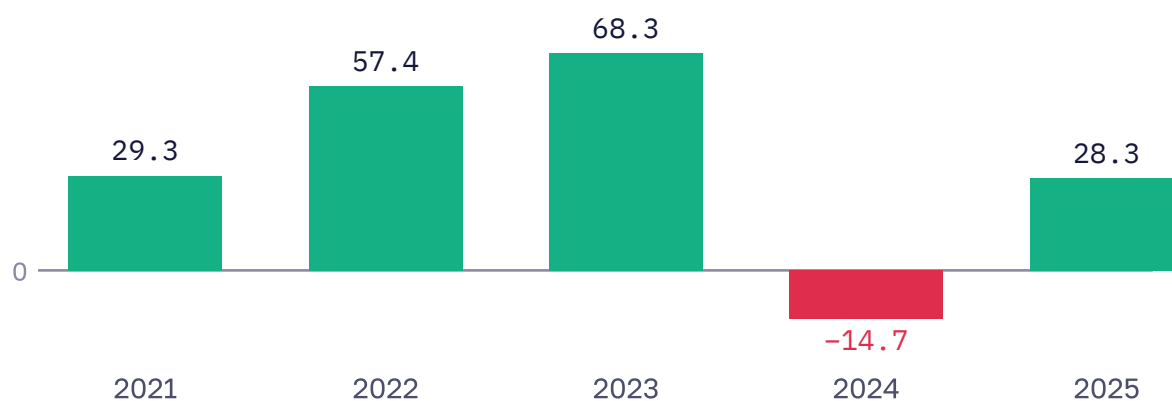
"On momentum metrics Lilly is clearly ahead – even as Novo holds the volume lead."

– the cycle / industry lens

What's priced in

On normalised earnings (assuming capex falls back toward depreciation), today's price implies just *~0–3% annual growth for a decade* at an 8–10% cost of equity – a pessimistic path for a 21.7% historical compounder. But it's conditional: anchor on today's depressed free cash flow and the same price implies +6–20%, i.e. the price already requires a big cash-flow recovery. So the low implied expectation is an opportunity only if the capex build-out normalises.

Exhibit 2: Free cash flow went negative in 2024 – the valuation needs it to recover



Source: Novo Nordisk AR 2025 "Five-year overview" (free cash flow, DKK bn; 2024 = -14.7). Retrieved 6 July 2026.

Through different investing lenses (perspectives, not a scorecard)

Quality: elite signature, but every metric turned down in 2025; the moat is real yet narrowing. **Cycle:** sentiment swung euphoria→despair – a contrarian setup, if the analysis holds. **Valuation:** undemanding, but conditional on the cash-flow recovery. **Inversion/risk:** the sharpest risk is becoming "the leader who loses the head-to-head and doesn't recover"; mitigants are incumbent share, breadth beyond one molecule, and the dividend. (*Perspectives to weigh – not ratings, not a recommendation.*)

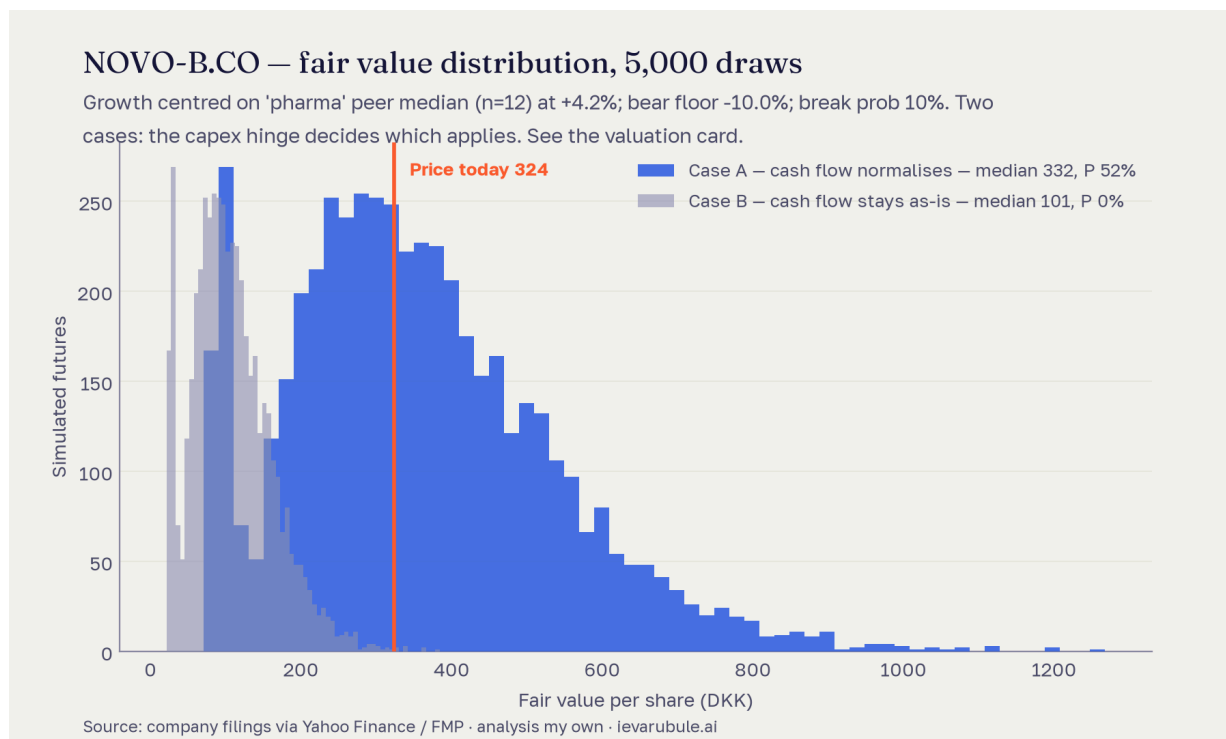
"The price already embeds the bear case – the question, for you to judge, is whether the bear case is right."

– the valuation lens

The valuation, run 5,000 times

Read the price backwards (a reverse-DCF) and DKK 324 requires roughly *1% profit growth a year for a decade*. Against its twelve actual pharma peers that is stark: the group median grew ~4%/yr, Novo's own record was the *second fastest of the twelve* – yet the growth its price implies would rank *tenth*. So I ran the valuation five thousand times, growth drawn around the peer median with a real bear floor and structural-break scenarios included. The answer splits on one assumption:

Exhibit 3: 5,000 simulated futures – the gap between the two humps is the capex assumption



Source: my calculation — seeded Monte Carlo, 5,000 draws per case; earnings bases spot-checked to AR 2025 (filed free cash flow DKK 28,989m, p.84). [Inferred]

Case A — cash flow normalises toward profit (as management guides): fair value clears today's price in *≈52% of simulations* — roughly even odds under Case A's assumptions.

Case B — cash flow stays as-is: it clears in *≈0%*. Neither number means anything without the other: the share price is not really the question, the capex hinge is.

The questions that decide it — you draw the conclusion

I keep my own buy/sell/hold decision private, deliberately: it depends on *my* circumstances and portfolio, which aren't yours, and a public "call" is both less useful and more fraught than showing you how to reason about it. So here are the questions I think it comes down to — and only your own honest answers to them can settle it for you.

1. **Do you understand this industry well enough** to judge pipeline and pricing risk — or is it honestly "too hard" for you?
 2. **Cyclical or structural?** Was 2025–26 a temporary dent (capacity build-out, accounting/pricing noise) or a permanent worsening of the competitive position?
 3. **Do you already own this inside a broad index fund?** If so, does an active overweight clearly earn its place versus simply staying indexed?
 4. **What specifically would change your mind** — in either direction — and are you watching for it?

What I'm watching (things to monitor, not instructions)

SIGNAL	WHY IT MATTERS
US Wegovy weekly prescription share vs Zepbound	The earliest tell on the franchise
2026 vs guidance; whether 2027 sales growth inflects	Cyclical-vs-structural resolves here
Whether capex declines and free cash flow recovers	Whether the recovery (Case A) assumption holds
Whether the Medicare price cut spreads to commercial	Depth of the pricing reset

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